جوستاف روسي انترناشيونال - مصر (EPE) *

طلب علاج أول مرة / متردد

إسم المريض رباعياً : ناصره احمد حسن احمد

المحافظة : القاهره **الحي / المدينة :** الازبكيه **العنوان :** 3 ح الطرابيشي

رقم البطاقة : 26711150103969 **رقم التليفون :** 01115188478 **تاريخ تسجيل الإستمارة :** 21-10-2026

العمر(تاريخ الميلاد) : 15-11-1967 **المهنة :** ربه منزل

الإقرار

أقر انا الموقع أدناه انني لا اتمتع باي نظام علاج مثل:-التأمين الصحي-القوات المسلحه-الشرطة-الهيئات-المؤسسات او اي نظام يتعارض مع نظام العلاج علي نفقة الدولة- وسوف اتحمل كافة نفقات العلاج في حالة ثبوت غير ذلك

الإخصائي الاجتماعي **المقر بما فيه**

التوقيع:(----------) **التوقيع:**(----------)

التشخيص الإكلينيكي : سرطان بالثدي (مبادرة)

نتائج الأبحاث : (مرفق)

التشخيص النهائي : سرطان بالثدي (مبادرة)

خطة العلاج : علاج مبادرة سرطان الثدي علاج تدعيمي [4953] **قسم العلاج :** الاورام

التعليق على التشخيص : ----------

تقرير اللجنة الثلاثية المتخصصة

الاسم: د/محمد كيلاني **الوظيفة:** ---------- **التوقيع:** ----------

الاسم: د/ندى حسن **الوظيفة:** ---------- **التوقيع:** ----------

الاسم: د/نرمين رمضان **الوظيفة:** ---------- **التوقيع:** ----------

أطلعت اللجنة علي لائحة العلاج الخاصة بالمجالس وتلتزم جهة العلاج بلائحة المجالس الطبية. كما تقر اللجنة بأن البيانات المدونة فى هذا النموذج صحيحة.

تعتمد صحة الاجابات المرفقة رئيس اللجنة و مدير المستشفي.

تتعهد المستشفى أن لا يتم تنفيذ القرار إلا بعد مراجعة الملف الطبي للمريض وإعادة مناظرة المريض بالمستشفي قبل الصرف.

أعضاء اللجنة

ا.د/ ---------- **ا.د/** ---------- **د/** ---------- **د/** ----------

قرار اللجنة

التشخيص النهائي : ----------

خطة العلاج : ----------

المستشفي : ----------**قسم :** ----------

جهة العلاج : ----------**نفقات :** ----------

ملاحظات : ----------

رئيس	مدير عام	ختم
المجلس الطبي المتخصص	مديرية الشئون الصحية	شعار الجمهورية

Name: نامره احمد حسن احمد
National ID: 26711150103969

GUSTAVE ROUSSY

تقرير طبي

السيد

تحية طيبة وبعد,,

إيماء للطلب المقدم من	بشأن تقرير طبي للمريض
بقسم:	رقم طبي:

نتشرف بأن نوضح فيما يلي تقريرا عن حالته

تاريخ ومساعة الدخول : ---:-- yyyy-mm-dd	تاريخ ومساعة الخروج : PM 01:17 05-07-2026

وبعد الكشف والفحوصات والمتابعة وجد انة

المريضة تعاني من ورم غير حميد بالثدى و تحتاج الي Supportive pharmacologic treatment with investigations

التاريخ : 05-07-2026	الوقت : -- --:--
	الطبيب المعالج : د/شيماء لاشين

Patient's Name: Nasra Ahmed Hassan

Date: 2026-08-13

Referred by: Prof Dr. no doctor

Hospital: Gonstave Roussey International Egypt

Ultrasonographic examination of the neck, revealed:

- The thyroid gland is not enlarged with a homogenous, parenchymal echotexture.
  - a. **Right Lobe:** measuring about 1.4 cm in AP, and show small nodule. It is located in the lower pole, measuring 0.9 X 0.8 X 0.8 cm, mixed cystic and solid and isoechoic with smooth margins and no calcification.
  - b. Left Lobe: measuring about 1.7 cm in AP dimeter, with no suspicious focal lesion.
  - c. **Isthmus:** Unremarkable in thickness and normal in echotexture. No focal isthmic lesions.
- Bilateral parotid and submandibular glands are symmetrical, normal in size, and display homogeneous echotexture. No focal lesions, ductal dilatation, or inflammatory changes are seen.
- Large left supra clavicular heterogenous lesions, largest about 3.5 X 2 cm, and this lesions demonstrates internal vascularity.
- Bilateral small cervical lymph nodes are identified bilaterally.
- The carotid arteries and internal jugular veins are patent bilaterally.

Conclusion:

- Right lobe dominant nodule: ACR TI-RADS 2 / 3. Follow-up ultrasound is recommended.
- Pathological lesions are seen within the left supra-clavicular region, as described, likely metastatic lymph nodes, for histopathology correlation.

Best regards;

Dr. Mohammad Islam

Patient Name	: الأستاذة / نصرة احمد حسن	Request No	: 13007861
Patient No.	: 28711150103969	Request Date	: 27/05/2023
Path No.	: 1602/23	Verification Date	: 06/06/2023
Sex	: Female	Financial Category	: برنامج دار السلام - تغطية كاملة
Age	: 55 Y	Department	: Out Patient
Referring Doc.	:	IHC No.	:
		Registered by	: marwa mohamed yousse

IMMUNOHISTOCHEMISTRY REPORT

Clinical Data:
Rt breast mass lesion
Nature of the specimen:
Tru cut biopsy collected 25-5-2023 (1:25PM)
Gross Examination:
Four greyish white tissue cores up to 1.5 cm . Totally submitted .
Sections prepared from paraffin block for immune-staining using Bench Mark ULTRA system for ER, PR,HER2, ER clone (1F2), PR clone (SP1), HER-2 clone (4B5), & Ki67 clone (30-9)

Microscopic examination:
Histological type: Invasive duct carcinoma
Grade (differentiation+ nuclear pleomorphism + mitosis) G II(3+2+1)
Ductal carcinoma in situ: Focal (Solid pattern)
IHC :
ER: Strong nuclear reaction in 80% of tumor cells (Allred score 8/8)
PR: Strong nuclear reaction in 80% of tumor cells (Allred score 8/8)
Her-2: No membrane reaction in tumor cells (score 0)
Ki67: 25 % nuclear reaction in tumor cells

Diagnosis:
Rt breast mass lesion , Tru cut biopsy:
Invasive duct carcinoma grade II
ER : Positive (8/8)
PR : Positive (8/8)
HER2 : Negative (score0)
Ki 67: 25 % nuclear reaction in tumor cells

Verified by :
dr.esraa magdy

Patient Name	:الأستاذه / ناصرة احمد حسن	Request No.	: 13009972
Patient No.	: 26711150103969	Request Date	: 16/09/2025
Path No.	: 2863/25	Verification Date	: 01/10/2025
Sex	: Female	Financial Category	: جوستاف روسي - تغطية الدولة
Age	: 57 Y	Department	: Out Patient
Referreing Doc :		Registered by	: Mrs.Mayada Hegazy

Pathology Report

CLINICAL DATA
Left supraclavicular lymph node enlargement .

NATURE OF SPECIMEN
Tru cut biopsy, collected on 17/9/2025.

GROSS EXAMINATION
Multiple grayish white tissue cores ranging insize from 0.3 up to 0.5 cm, totally submitted.
IHC technique: Sections prepared from paraffin block for immune-staining using Bench Mark ULTRA system for ER, PR, HER2, ER clone (1E2), PR clone (SP1), HER-2 clone (4B5), & Ki67 clone (30-9),

MICROSCOPIC EXAMINATION
* Histological type: Metastatic carcinoma of breast origin
* Grade (differentiation +nuclear pleomorphism+ mitosis): (3+2+2) Grade 2

• IPT (Positive control showed appropriate reactivity) :
*GATA3 : Diffuse positive
* ER: No nuclear reaction in tumor cells (Allred score 0/8)
* PR: No nuclear reaction in tumor cells (Allred score 0/8)
* HER2: Moderate compelete membrane reaction in >10 % of tumor cells (score 2+)
-SISH : Not amplified.

DIAGNOSIS
Left supraclavicular lymph node , Tru cut needle (TCNB) biopsy:
Metastatic carcinoma of mammary origin

Verified by :
Dr.Heba Hasan

غرفة رقم (4) – الدور الأول – مبنى رقم (987) – كورنيش النيل – مصر القديمة – القاهرة – مصر
Room No. (4) - First Floor-Building No. 987- Nile Corniche - Misr El Qadema - Cairo, Egypt

Pathology Report
[DOCTYPE:PATHOLOGY_REPORT]

ISTAVE
USSY

Patient Name	: الأستاذه / نصرة احمد حسن	Request No.	: 13009972
Patient No.	: 26711150103969	Request Date	: 18/09/2025
Path No.	: 2863/25	Verification Date	: 01/10/2025
Sex	: Female	Financial Category	: جوستاف روسي - نفقة الدولة
Age	: 57 Y	Department	: Out Patient
Referreing Doc	:	Registered by	: Mrs.Mayada Hegazy

Pathology Report

IPT:
* ER: Negative (score 0/8).
* PR: Negative (score 0/8).
* HER2: Equivocal (score 2+).
*SISH : -Not amplified
-Average Her2 copy No .2.7
-HER2/CEP17 ratio 1.2
* Scoring system: "ER, PR"
ASCO-CAP ER/PR Test Guideline Recommendations, 2010 Allred score: proportion of cells staining (0, no staining; 1, >1%; 2, between 1% and 10%; 3, between 11% and 33%; 4, between 34% and 66% and 5, between 67%-100% of the cells staining). Intensity of positive tumor cells (0, none; 1, week, 2, Intermediate; and 3, strong). Total score from 0 to 8.
* Scoring system: "Her2"
Reference: ASCO-CAP Her2 Test Guideline Recommendations, 2013.IHC 0: as defined by no staining observed or membrane staining that is incomplete and is faint/barely perceptible and within < 10% of the invasive tumor cells. IHC 1+: as defined by incomplete membrane staining that is faint/barely perceptible and within < 10% of the invasive tumor cells. IHC 2+: based on circumferential membrane staining that is incomplete and/or weak/moderate and within < 10%of the invasive tumor cells or complete and circumferential membrane staining that is intense and within < 10% of the invasive tumor cells. IHC 3+: based on circumferential membrane staining that is incomplete, intense < 10% of tumor cells.

Signature
Dr. Walaa Ibrahim Ghoneim
Dr. Heba El-shafeey

Verified by :
Dr.Heba Hasan

غرفة رقم (4) – الدور الأول – مبنى رقم (987) – كورنيش النيل – مصر القديمة – القاهرة – مصر
Room No. (4) - First Floor-Building No. 987- Nile Corniche - Misr El Qadema - Cairo, Egypt

Pathology Report
[[DOCTYPE:PATHOLOGY_REPORT]]

Ministry Of Health
Specialized Medical Centers
Dar El Salam Cancer Hospital
Radiology Department

وزارة الصحة والسكان
أمانة المراكز الطبية المتخصصة
مستشفى أورام دار السلام
إدارة الأشعة

Patient Name: Nasera Ahmed Hassan Ahmed	Date : 08/06/2023
Exam : MAMMOGRAPHY BOTH	Age : 55 Year(s)
Medical ID: 2015	

Combined bilateral breast mammography and complementary ultrasound

Indication:

left breast palpable lump.

Technique

- Bilateral MLO and CC views
- Bilateral breast ultrasound

Report:

- Scattered fibro glandular density (ACR B)
- Left breast shows UOQ area of asymmetrical density reaching the nipple measuring about 9x3.5 cm associated with skin edema corresponding by U/S to UOQ ill defined spiculated solid mass lesion located at 2 o'clock measuring about 5x4Sx25 cm showed anterior ductal extensions reaching the nipple and internal hypervascularity on Color Doppler.
- Right UOQ well defined isodense mass lesion corresponding by U/S to UOQ IMLN with preserved fatty hilum and average cortical thickness.
- Normal skin thickness, and nipple-areola complex of the right breast.
- Pathological left axillary lymph nodes are seen.
- Prominent left infra and supraclavicular lymph nodes.
- Nonspecific right axillary LN.

Opinion:

- Left breast UOQ locally advanced malignant looking mass lesion with pathological axillary lymph nodes(BIRADS 5) for histopathological correlation.
- Benign findings right breast (BIRADS 2)

Yours truly

Dr/Asmaa Zeinhom, Msc

Signature Temp

08/06/2023 2:32 PM

GUSTAVE ROUSSY
INTERNATIONAL

Patient Name : Nasera Ahmed Hassan Ahmed | Date : 27/08/2025
Exam : US Neck | Age : 57 Year(s)
Medical ID : | Pat ID : 38846

Neck Ultrasonography

- The right and left lobes and isthmus of thyroid gland are of normal size, homogenous echo pattern, with regular outline.
- Right thyroid lobe complex cystic and solid sub centimetric nodule (TIRADS 3)
- Normal vascularity at CCDI.
- The gland moves freely with deglutition.
- The trachea is centralized.
- Both carotid sheaths are free.
- No sizable cervical lymph nodes
- Left supraclavicular pathological LNS associated with smudging of the surrounding fat, the largest is measuring 9x6mm

Conclusions

Left supraclavicular pathological LNS for biopsy
Right thyroid lobe nodule (TIRADS 3) for follow up

Signature Dr . Nermien Ramadan
27/08/2025 12:26 PM

Ministry Of Health
Specialized Medical Centers
Dar El Salam Cancer Hospital
Radiology Department

وزارة الصحة والسكان
أمانة المراكز الطبية المتخصصة
مستشفى أورام دار السلام
ادارة الاشعة

Patient Name: Nasera Ahmed Hassan Ahmed	Date : 08/06/2023
Exam : MAMMOGRAPHY BOTH	Age : 55 Year(s)
Medical ID: 2015	

Combined bilateral breast mammography and complementary ultrasound

Indication:

left breast palpable lump.

Technique

- Bilateral MLO and CC views
- Bilateral breast ultrasound

Report:

- Scattered fibro glandular density (ACR B)
- Left breast shows UOQ area of asymmetrical density reaching the nipple measuring about 9x3.5 cm associated with skin edema corresponding by U/S to UOQ ill defined spiculated solid mass lesion located at 2 o'clock measuring about 5x45x25 cm showed anterior ductal extensions reaching the nipple and internal hypervascularity on Color Doppler.
- Right UOQ well defined isodense mass lesion corresponding by U/S to UOQ IMLN with preserved fatty hilum and average cortical thickness.
- Normal skin thickness, and nipple-areola complex of the right breast.
- Pathological left axillary lymph nodes are seen.
- Prominent left infra and supraclavicular lymph nodes.
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Opinion:

- Left breast UOQ locally advanced malignant looking mass lesion with pathological axillary lymph nodes(BIRADS 5) for histopathological correlation.
- Benign findings right breast (BIRADS 2)

Yours truly

Dr/Asmaa Zeinhom, Msc

Signature Temp

08/06/2023 2:32 PM

Mammogram
[DOCTYPE:MAMMOGRAM]

Ministry Of Health
Specialized Medical Centers
Dar El Salam Cancer Hospital
Radiology Department

وزارة الصحة والسكان
أمانة المراكز الطبية المتخصصة
مستشفى أورام دار السلام
ادارة الاشعة

Patient Name: Nasera Ahmed Hassan Ahmed	Date : 10/06/2023
Exam : C.T. ABDOMEN &PELIVS WITH CONRAST	Age : 55 Year(s)
DLP:762mGy.cm	Pat. ID: 38846 2112/2023

POST CONTRAST MULTI-SLICE CT SCAN OF THE CHEST, ABDOMEN & PELVIS REVEALED: -

Reported case of left breast cancer shows:

1. ***Chest:***

- Left breast heterogenous parenchyma with enlarged ipsilateral axillary lymph nodes, largest measures about 1.5X 1.2 cm with illdefined enhancing sheets
- Bilateral few atelectatic bands in the middle lobe and lingula. No sizable masses, nodules, consolidation patches or cavitary lesions.
- No evidence of pleural effusion or masses seen.
- Average cardiac size. No pericardial effusion detected.

2. ***Abdomen and pelvis:***

- The liver is average in size with homogenous CT texture. No definite focal lesions or contour irregularities.
- Patent homogenously enhancing portal vein and is main branches as well as its main tributaries.
- No intra-hepatic or extra-hepatic biliary radicles dilatation.
- Normal size & parenchymal texture of the spleen with no focal lesions seen.
- No pathological abdominal or pelvic lymph nodes.
- Normal size, parenchymal thickness & density of both kidneys with no stones or back-pressure changes.
- Normal appearance of the pancreas, aorta, I.V.C and both supra-renals.
- Smooth filling of the urinary bladder with no filling defects or diverticular outpouching.
- Bulky lobulated uterine appearance is seen.
- No abdominal or pelvic masses or collections.

3. ***The scanned skeleton:***

- The scanned skeleton shows no definite worrisome lytic or sclerotic lesions.

Patient's Name:	Nasra Ahmed Hassam
Date:	**2026-08-13**
Referred by:	Prof.Dr. **no doctor**
Hospital:	**Goustave Roussey International Egypt**

Ultrasonographic examination of the neck, revealed:

- The thyroid gland is not enlarged with a homogenous, parenchymal echotexture.
  - **Right Lobe:** measuring about 1.4 cm in AP, and show small nodule. It is located in the lower pole, measuring 0.9 X 0.8 X 0.8 cm, mixed cystic and solid and isoechoic with smooth margins and no calcification.
  - **Left Lobe:** measuring about 1.7 cm in AP dimeter, with no suspicious focal lesion.
  - **Isthmus:** Unremarkable in thickness and normal in echotexture. No focal isthmic lesions.
- Bilateral parotid and submandibular glands are symmetrical, normal in size, and display homogeneous echotexture. No focal lesions, ductal dilatation, or inflammatory changes are seen.
- Large left supra clavicular heterogenous lesions, largest about 3.5 X 2 cm, and this lesions demonstrates internal vascularity.
- Bilateral small cervical lymph nodes are identified bilaterally.
- The carotid arteries and internal jugular veins are patent bilaterally.

Conclusion:

- Right lobe dominant nodule: ACR TI-RADS 2 / 3. Follow-up ultrasound is recommended.
- Pathological lesions are seen within the left supra-clavicular region, as described, likely metastatic lymph nodes, for histopathology correlation.

Best regards;

Dr. Mohammad Islam

Patient's Name:	**Nasira Ahmed Hassan Ahmed**
Date:	**2026-08-20**
Referred by:	Prof.Dr. **no doctor**
Hospital:	**Goustave Roussey International Egypt**

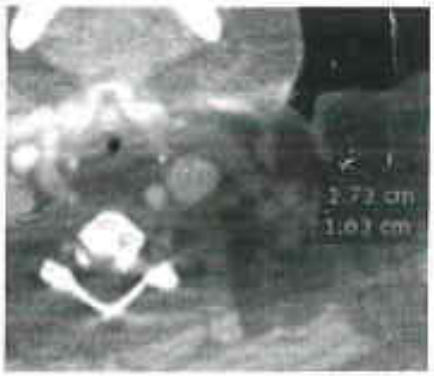
POST-CONTRAST MULTI-SLICE CT SCAN OF THE NECK, CHEST, ABDOMEN AND PELVIS

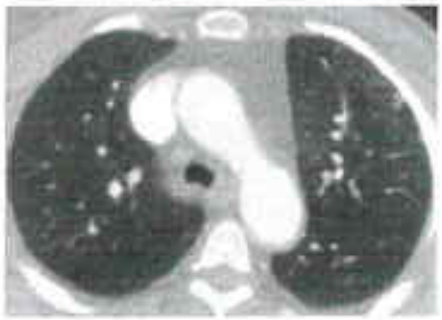
Known patient with left breast cancer. Compared to the previous study dated 21/6/2026:

I-NECK:

- The lower brain cuts taken are unremarkable.
- Normal appearance of the naso-, oro- and hypopharynx with preserved parapharyngeal fat planes.
- Intact laryngeal skeleton with no glottic, supra or infra glottic masses.
- Normal appearance of submandibular, parotid salivary glands.
- Normal size, enhancement & symmetrical appearance of thyroid glands.
- Normal course and caliber of the carotid and jugular vessels.
- Few subcentemetric bilateral upper deep cervical and submental LNs are noted; likely non-specific.
- No pathologically enlarged cervical lymph nodes.

II-CHEST:





- Clear left mastectomy bed by CT.
- **Beam hardening artifact hinders proper assessment of the supraclavicular region. The suspected left supraclavicular LN now measures 1.7x1.6 cm compared to 2x0.8 cm in the previous study? for US correlation.**
- Still noted patchy area of ground glass opacity in the anterior segment of left upper pulmonary lobe and linear atelectatic bands in the middle lobe and in the lingula.
- Still noted middle lobe patch of consolidation.
- No sizable suspicious pathological enlarged lymph nodes.
- No evidence of pleural effusion, pleural thickening or masses seen.
- Mild cardiomegaly. No pericardial effusion detected.
- Normal enhanced appearance of the thoracic aorta and the great vessels.

Patient's Name:	**Nasira Ahmed Hassan Ahmed**
Date:	**2026-08-20**
Referred by:	Prof.Dr. **no doctor**
Hospital:	**Goustave Roussey International Egypt**

- Right hypodense thyroid nodule for US correlation.

III-ABDOMEN AND PELVIS

- Average size of the liver showing homogenous low parenchymal attenuation likely fatty. No focal lesion or dilated biliary radicles.
- Patent homogenously enhanced portal vein and its main branches.
- Normal CT appearance of both kidneys showing normal site, size and shape with smooth regular outline. No definite cysts, masses or back pressure changes.
- Normal C.T appearance of the spleen, adrenal glands, pancreas, and IVC.
- Atherosclerotic changes of the aorta.
- The urinary bladder is empty.
- Bulky uterus for age.
- No suspicious abdominal or pelvic lymphadenopathy.
- No ascites.
- Average caliber of the bowel loops.
- Bone window reveals no suspicious lytic or sclerotic lesions.

OPINION :

- **Follow up study of a patient with history of left breast cancer, current study reveals rather stationary findings.**
- **The previously suspected left supraclavicular LN is better to be assessed by US due to beam hardening artifact.**

Shohenda. M. Taha, MD

CT Scan
[DOCTYPEST_SCAN]

Patient's Name:	Nasra Ahmed Hassam
Date:	**2026-08-13**
Referred by:	Prof.Dr. **no doctor**
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Patient's Name:	**Nasira Ahmed Hassan Ahmed**
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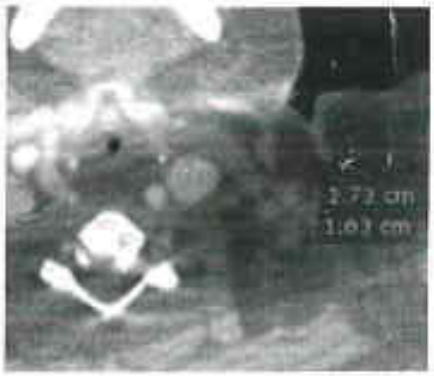
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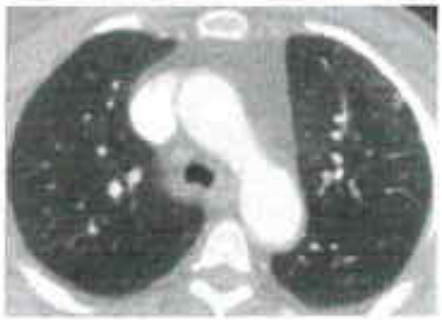
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Patient's Name:	**Nasira Ahmed Hassan Ahmed**
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Shohenda. M. Taha, MD

جوستاف روسي

Patient's Name: Nasra Ahmed Hassam
Date: **2026-08-13**
Referred by: Prof.Dr. **no doctor**
Hospital: **Goustave Roussey International Egypt**

Ultrasonographic examination of the neck, revealed:

- The thyroid gland is not enlarged with a homogenous, parenchymal echotexture.
  - **Right Lobe:** measuring about 1.4 cm in AP, and show small nodule. It is located in the lower pole, measuring 0.9 X 0.8 X 0.8 cm, mixed cystic and solid and isoechoic with smooth margins and no calcification.
  - **Left Lobe:** measuring about 1.7 cm in AP dimeter, with no suspicious focal lesion.
  - **Isthmus:** Unremarkable in thickness and normal in echotexture. No focal isthmic lesions.
- Bilateral parotid and submandibular glands are symmetrical, normal in size, and display homogeneous echotexture. No focal lesions, ductal dilatation, or inflammatory changes are seen.
- Large left supra clavicular heterogenous lesions, largest about 3.5 X 2 cm, and this lesions demonstrates internal vascularity.
- Bilateral small cervical lymph nodes are identified bilaterally.
- The carotid arteries and internal jugular veins are patent bilaterally.

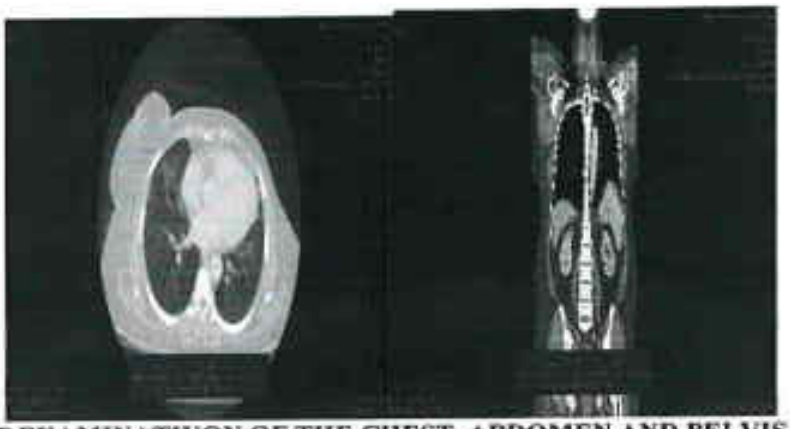
Conclusion:

- Right lobe dominant nodule: ACR TI-RADS 2 / 3. Follow-up ultrasound is recommended.
- Pathological lesions are seen within the left supra-clavicular region, as described, likely metastatic lymph nodes, for histopathology correlation.

Best regards;

Dr. Mohammad Islam

Patient's Name:	**Nasira Ahmed Hassan Ahmed**
Date:	**2026-06-21**
Referred by:	Prof.Dr. **no doctor**
Hospital:	**Goustave Roussey International Egypt**



MULTISLICE CT EXAMINATIUON OF THE CHEST ,ABDOMEN AND PELVIS WITH CONTRAST ADMINISTRATION REVEALED:-

- Evident left breast mastectomy with clear operative bed and no evidences of either residual and /or recurrent neoplastic lesions seen herein.
- Normal right breast.
- Relatively reduced the size of the left supra-clavicular lymph node ,measuring about 20x8.2mm in dimensions(versus 21x12mm in the previous study done at 11/03/2026).
- Hyperinflated both lungs.
- Persistence of the patchy area of ground glass opacity in the anterior segment of left upper pulmonary lobe and linear atelectatic bands in the middle lobe and in the lingula and a newly-developed ground-glass opacity also seen in the middle lobe.
- No sizable abnormally-enhancing pulmonary nodules or foci of calcifications noted.
- No sizable mediastinal or hilar lymph nodes seen.
- Patent central airways.
- No pleural or pericardial effusion.
- Border line size of the heart with atherosclerotic changes of its big vessels.
- **Intact bony thoracic cage apart from spondylo-degenerative changes of the scanned spine. No lytic or sclerotic bony lesions seen.**
- Border line sized liver,measuring about 15.9 cm, showing a homogenous parenchymal attenuation and enhancement pattern.
- No sizable abnormally-enhancing hepatic focal lesions or dilated biliary radicles.
- Patent PV.
- Average sized spleen,measuring about 11.8 cm. No focal lesions seen inside.
- Thickened both adrenal glands.
- Normal CT appearance of the pancreas, functioning both kidneys, aorta and IVC.
- No sizable abdominal or pelvic lymphadenopathy.
- No ascites.
- An irregular mucosal thickening of the gastric wall=18 mm.
- No intra-vesical stones,solid masses, or diverticulosis seen. An irregular thickening of the vesical wall is seen and reaches to about 5.9mm.
- Bulky uterus.
- The rest of the pelvic organs are unremarkable.

IMPRESSIONS:

Patient's Name:	**Nasira Ahmed Hassan Ahmed**
Date:	**2026-06-21**
Referred by:	Prof.Dr. **no doctor**
Hospital:	**Goustave Roussey International Egypt**

- **Evident left breast mastectomy with clear operative bed and no evidences of either residual and /or recurrent neoplastic lesions seen herein.**
- **Relatively reduced the size of the left supra-clavicular lymph node ,denoting a partial regressive course.**
- **Thickened both adrenal glands,likely to be hyperplasia rather than more serious pathology.**
- **Hyperinflated both lungs,with persistence of the patchy area of ground glass opacity in the anterior segment of left upper pulmonary lobe and linear atelectatic bands in the middle lobe and in the lingula,denoting a stable course and a newly-developed ground-glass opacity also seen in the middle lobe.**
- **An irregular mucosal thickening of the gastric wall,denoting a gastritis.**
- **An irregular thickening of the vesical wall ,denoting a cystitis.**
- **Bulky uterus.For TVUS study.**
- **Otherwise ,normal CT of the chest,abdomen and pelvis with contrast.**

Prof/Hanan El-Ahwal.

GUSTAVE ROUSSY
INTERNATIONAL CANCER

Name : nasera ahmed hassan ahmed	Date : 11-03-2026 11:06
Branch : Portable	Medical No. : GRI-38846
Ref.Physician Prof : NCAP	Ins.Company :

MULTISCLICE POST CONTRAST C.T. EXAMINATION OF THE NECK, CHEST, ABDOMEN AND PELVIS REVEALED:

Compared to the previous study dated 1/11/2025, the current study showed:

I- NECK:

- Rather stationary course regarding the size of the previously seen left supraclavicular lymph node measuring 21 x 12 mm.
- Normal appearance of the naso-, oro- and hypopharynx with preserved parapharyngeal fat planes.
- Intact laryngeal skeleton with no glottic, supra or infra glottic masses.
- Normal course and calibre of the carotid and jugular vessels.
- Normal CT appearance of the submandibular and parotid salivary glands showing homogenous enhancement. No focal solid or cystic lesions.
- Few subscentemetric bilateral upper deep cervical and submandibular LNs are noted; likely non-specific.

II- CHEST:

- Evidence of left mastectomy.
- Mild progressive course regarding the previously seen left upper lobe supleural reticulations likely post radiation sequelae.
- Still noted bilateral scattered atelectatic bands.
- No pulmonary nodules, masses or cavitary lesions.
- No consolidation or collapse.
- No hilar or mediastinal masses or lymph node enlargement.
- No pleural or pericardial collection.
- Normal CT appearance of the heart and great vessels.

III- ABDOMEN AND PELVIS:

- Enlarged liver showing homogenous parenchymal attenuation. No hepatic focal lesion or dilated biliary radicles.
- Both kidneys are normal site, average sized and showing smooth regular cortical outline and preserved parenchymal thickness and function. No stones or back pressure changes.

GUSTAVE ROUSSY
INTERNATIONAL

Name : nasera ahmed hassan ahmed	**Date : 11-03-2026 11:06**
Branch : Portable	**Medical No. : GRI-38846**
Ref.Physician Prof : NCAP	**Ins.Company :**

- No appreciable time interval changes regarding the previously seen bilateral adrenal nodular thickening.
- Normal C.T appearance of the spleen, pancreas, aorta and IVC.
- The urinary bladder shows no evidence of gross masses or diverticulae or stones.
- Still noted bulky uterus.
- Normal appearance of the rest of pelvic organs with no pelvic masses or collections.
- No abdominal or pelvic lymphadenopathy.
- No ascites.

Opinion:

❖ **Rather stationary course since last study.**

Dr. Sherif Wagdy

Dr. Sherif Wagdy

USTAVE
OUSSY
RNATIONAL

tient Name: Nasera Ahmed Hassan Ahmed	Date : 01/11/2025
am : CT NCAP +C(CT Neck Chest Abd. Pelvis)	Age : 58 Year(s)
edical ID: 2112/2023	Pat. ID: 38846

ST CONTRAST MULTI-SLICE CT SCAN OF THE NECK ,CHEST, ABDOMEN & PELVIS

REVEALED: -

ported case of left breast cancer , compared to last study 10/6/2023 , the current idy shows:

ck

Normal appearance of the nasopharynx, oropharyngeal and laryngeal air spaces.
No glottic, supra or infra glottic mass lesions.
Intact cartilaginous skeleton of the larynx.
Normal appearance of submandibular, parotid salivary glands.
Normal size and symmetrical appearance of thyroid glands.
Prominent subcm cervical lymph nodes in their short axes.

iest:

Left mastectomy with clear operative bed , no masses or collection.
Left upper lobe subpleural reticulation likely post radiotherapy .
Still seen bilateral few atelectatic bands in the middle lobe and lingula. No sizable masses, nodules, consolidation patches or cavitary lesions.
No evidence of pleural effusion or masses seen.
Average cardiac size. No pericardial effusion detected.

bdomen and pelvis:

The liver is average in size with homogenous CT texture. No definite focal lesions or contour irregularities.
Patent homogenously enhancing portal vein and is main branches as well as its main tributaries.
No intra-hepatic or extra-hepatic biliary radicles dilatation.
Normal size & parenchymal texture of the spleen with no focal lesions seen.
No pathological abdominal or pelvic lymph nodes.
Normal size, parenchymal thickness & density of both kidneys with no stones or back-pressure changes.
- Normal appearance of the pancreas, aorta, I.V.C
- **Both supra-renals are showing nodular thickening at current study..**
- Smooth filling of the urinary bladder with no filling defects or diverticular outpouching.
- Bulky lobulated uterine appearance is still noted for dedicated US study..
- No abdominal or pelvic masses or collections.

he scanned skeleton:

- The scanned skeleton shows no definite worrisome lytic or sclerotic lesions.
- No vertebral structural collapse or extra-osseous soft tissue component.

OPINION:

Reported case of left breast cancer

Bilateral suprarenal nodular thickening
Otherwise stationary course is noted .

Signature Reeham.Khader
03/11/2025 10:50 AM

Name : Nasra Ahmed Hassan
Branch : Dokki
Ref.Physician Prof : Gostaf Roseh Hospital

Date : 23-10-2025 11:07
Medical No. : 231043
Ins.Company : تعاقد مستشفيات

$Tc^{99}m$ MDPSKELETAL SCINTIGRAPHY

History

- A 58-year-old patient, with history of left breast cancer and referred for metastatic workup.

Technique

- Anterior and posterior whole-body scans were acquired 2 hours post injection of the tracer.

Findings

- No abnormal active or cold focal bony lesions that could account for metastatic bone disease.
- Degenerative changes are noted at the spine and big joints.

Impression:

- **The skeleton is free from metastases.**

Best regards,
Dr. Alaa Nagi, MD
Nuclear Medicine

جوستافروسي

مبادرة رئيس الجمهورية لدعم صحة المرأة المصرية

أسبوع صحة مصر

MDT FORM 11 / 10 / 2025 G

Name	ناصرة أحمد حسن	MRN	C11C
		ID	C.C4
Age	٥٧ سنة	Phone	01095172201
Menopausal status	Post	Marital status	
NRS SCORE		Family history	
BMI		REASON OF MDT	
CO morbidity	HTN - D.M		

Clinical Data

BX: IDC

ER:	PR:	HER 2:	Ki67:
8/8	8/8	-ve	

R L



Sonomammography:
2023 : AC x 4
TXT x 11

Mets Workup:
~~RT~~ MRM (T1 N3 M0 (14/25) PPR)
Adj : PORT IM

TNM
PS I

Recommendations:
AI (2.2.2024 till now)
U/S 9/2025: supra clavicular LN
Bx : met. carcinom of mammary origin.

MDT OUTCOME & PLAN (ER 8/8 = PR 8/8 - her2 -ve)
CT : (R) Bones (R) المريضة ظهر للتطبيق
~~Jardiance~~ Zoladex 3 cycles Recorded
~~for Letrozole weekly~~

SIGNATURE					
MEDICAL ONCOLOGY	SURGERY	RADIOLOGY	PATHOLOGY	Nutrition	Psychiatric





ERM036726182

صيادرة

Presidential Program for Women's Health
Ministry of Health and Population

البرنامج الرئاسي لصحة المرأة
وزارة الصحة والسكان

MDT form

A-Patient ID:
1- Name : نصره احمد حسن احمد Sex: Female
3- Medical number: 83146213
Old Med : 2112/2023
2-ID number: 26711150103969
Patient Age : 56 Y

Family history	-VE	Phoria		BMI	30	REASON OF MDT	NEW
marital status	MARRID	NRS Score	2	GROUP	A	Date of MDT	2023-06-21
Co morbidity	HTN -DM CARDIAC	Menopausal status	POSTMENOPAUSAL				

Clinical data

Breast Imaging :

Mets Workup :

BX:	ER:	PR:	Her 2 Neu :	SISH :	Ki67 :
IDC LT GII	8/8	8/8	0		25

T	N	M
4B	2	0

Sonomammography
LT UOQ+LT PLN+SKIN EDEMA + PROMINEN SUPRA& IFRA CLAV LN

PATIENT HISTORY
FOR ECHO
BONE SCAN

MDT OUTCOME & PLAN

AFTER CHECKING THE ECHO , FOR NACT 4 AC & 4 TAXANES THEN LEFT MRM

SIGNATURE

MEDICAL ONCOLOGY	د/ اشرف صقر	RADIOLOGY	د/
SURGERY	د/ محمود عبدالمنعم	PATHOLOGY	
Psychiatric		Nutrition	

Presidential Program for Women's Health
Ministry of Health and Population

البرنامج الرئاسي لصحة المرأة
وزارة الصحة والسكان

MDT form

A-Patient ID:
1- Name : نصره احمد حسن احمد Sex: Female
3- Medical number: 83:46213
Old Med : 2112-2023

2-ID number: 26711150103969 Patient Age : 56 Y

Family history	Family history -VE	Phone	Phone	BMI	BMI 29	REASON OF MDT	REASON OF MDT POST NACI
marital status	Marital Status MARRIED	NRS Score	NRS Score 2	GROUP	GROUP A	Date of MDT	Date Of MDT 13/12/2023
Co morbidity	Co Morbidity HTN -DM CARDIAC	Menopausal #atus	Menopausal Status POSTMENOPAUSAL	Initial STAGE	Initial STAGE		

Clinical data

Breast Imaging :

Mets Workup :

BX	ER	PR	Her 2 Neu	SISH	Ki67
IDC	8/8	8/8	0		25
BX COMMENT:	ER COMMENT:	PR COMMENT:	Her 2 Neu COMMENT:	SISH COMMENT	

T	N	M
4b	2	0

Sonomammography
LT UOQ+LT PLN+SKIN EDEMA + PROMINEN SUPRA& IFRA CLAV LN

PATIENT HISTORY
ended neoadj cth last 29/11

MDT OUTCOME & PLAN

MDT OUTCOME & PLAN
left MRM(initially T4bN2M0 luminal A breast IDC,NOW COMPLETE CLINICAL RESPONSE BOTH IN BREAST AND

SIGNATURE

MEDICAL ONCOLOGY	MEDICAL ONCOLOGY د/ شيماء لاشين	RADIOLOGY	RADIOLOGY د/ ايمان على
SURGERY	SURGERY د/ هبه جبل	PATHOLOGY	PATHOLOGY
Psychiatric	Psychiatric	Nutrition	Nutrition اسراء حربي

Ministry Of Health
Specialized Medical Centers
Dar El Salam Cancer Hospital
Radiology Department

وزارة الصـــــحة والسكـــان
أمانة المراكز الطبية المتخصصة
مستشفى أورام دار السلام
ادارة الاشعة

Patient Name: Nasera Ahmed Hassan Ahmed	Date : 08/06/2023
Exam : MAMMOGRAPHY BOTH	Age : 55 Year(s)
Medical ID: 2015	

Combined bilateral breast mammography and complementary ultrasound

Indication:

left breast palpable lump.

Technique

- Bilateral MLO and CC views
- Bilateral breast ultrasound

Report:

- Scattered fibro glandular density (ACR B)
- Left breast shows UOQ area of asymmetrical density reaching the nipple measuring about 9x3.5 cm associated with skin edema corresponding by U/S to UOQ ill defined spiculated solid mass lesion located at 2 o'clock measuring about 5x45x25 cm showed anterior ductal extensions reaching the nipple and internal hypervascularity on Color Doppler.
- Right UOQ well defined isodense mass lesion corresponding by U/S to UOQ IMLN with preserved fatty hilum and average cortical thickness.
- Normal skin thickness, and nipple-areola complex of the right breast.
- Pathological left axillary lymph nodes are seen.
- Prominent left infra and supraclavicular lymph nodes.
- Nonspecific right axillary LN.

Opinion:

- Left breast UOQ locally advanced malignant looking mass lesion with pathological axillary lymph nodes(BIRADS 5) for histopathological correlation.
- Benign findings right breast (BIRADS 2)

Yours truly

Dr/Asmaa Zeinhom, Msc

Signature Temp

08/06/2023 2:32 PM

Ministry Of Health
Specialized Medical Centers
Dar El Salam Cancer Hospital
Radiology Department

وزارة الصحة والسكان
أمانة المراكز الطبية المتخصصة
مستشفى أورام دار السلام
ادارة الاشعة

Patient Name: Nasera Ahmed Hassan Ahmed	Date : 10/06/2023
Exam : C.T. ABDOMEN &PELIVS WITH CONRAST	Age : 55 Year(s)
DLP:762mGy.cm	Pat. ID: 38846 2112/2023

POST CONTRAST MULTI-SLICE CT SCAN OF THE CHEST, ABDOMEN & PELVIS REVEALED: -

Reported case of left breast cancer shows:

1. ***Chest:***

- Left breast heterogenous parenchyma with enlarged ipsilateral axillary lymph nodes, largest measures about 1.5X 1.2 cm with illdefined enhancing sheets
- Bilateral few atelectatic bands in the middle lobe and lingula. No sizable masses, nodules, consolidation patches or cavitary lesions.
- No evidence of pleural effusion or masses seen.
- Average cardiac size. No pericardial effusion detected.

2. ***Abdomen and pelvis:***

- The liver is average in size with homogenous CT texture. No definite focal lesions or contour irregularities.
- Patent homogenously enhancing portal vein and is main branches as well as its main tributaries.
- No intra-hepatic or extra-hepatic biliary radicles dilatation.
- Normal size & parenchymal texture of the spleen with no focal lesions seen.
- No pathological abdominal or pelvic lymph nodes.
- Normal size, parenchymal thickness & density of both kidneys with no stones or back-pressure changes.
- Normal appearance of the pancreas, aorta, I.V.C and both supra-renals.
- Smooth filling of the urinary bladder with no filling defects or diverticular outpouching.
- Bulky lobulated uterine appearance is seen.
- No abdominal or pelvic masses or collections.

3. ***The scanned skeleton:***

- The scanned skeleton shows no definite worrisome lytic or sclerotic lesions.

Patient Name: Nasera Ahmed Hassan Ahmed	Date : 10/06/2023
Exam : C.T. ABDOMEN &PELIVS WITH CONRAST	Age : 55 Year(s)
DLP:762mGy.cm	Pat. ID: 38846 2112/2023

- No vertebral structural collapse or extra-osseous soft tissue component.

OPINION:

Reported case of left breast cancer

- ***Left breast malignant process with enlarged ipsilateral axillary lymph nodes and related soft tissue sheets.***
- ***No definite pulmonary, hepatic or osseous metastatic deposits.***

Signature Prof. Menna Tallah Hanafy
12/06/2023 05:47

وزارة الصحة والسكان
ادارة المراكز الطبية المتخصصة
مستشفى اورام دار السلام
قسم الايكو

Echo report

MR : ٩١٢/٢٠

Name: ناصرة احمد حسن

Date: 16/12/2023

EF	69 %	55-75%	PWT	0.36	
LVEDD	4.99	3.5-5.8cm	AO.R	1.90	2-3.7cm
LVESD	3.04	2-4cm	LA	3.44	2-4cm
FS	34 %	25-45%	RV	[illegible]	0.7-2.3cm(supine)
SWT	.90	0.6-1.1cm	AO.CUSP	—	1.5-2.6cm

LV dimensions Normal

LV wall thickness Normal

LV diastolic function D.D I

SWMA No regional wall motion abnormality

LA and Aortic root dimensions Normal

Mitral Valve Normal , No PG

Aortic valve A.S sclerotic, sclerotic AR

Tricuspid valve Trivial TR

Pulmonary valve No PG

Right side and systolic function Not dilated , TAPSE 19mm

Intracardiac masses and effusion No effusion, No masses

D/ Msc cardiology

دكتور احمد

Patient Name	: الأستاذه / ناصرة احمد لبسن	Request No.	: 13005369
Patient No.	: 26711150103969	Request Date	: 30/12/2023
Path No.	: 4608/23	Verification Date	: 15/01/2024
Sex	: Female	Financial Category	: اورام دار السلام - نفقة الدولة
Age	: 55 Y	Department	: Out Patient
Referreing Doc :		Registered by	: .marwa mohamed youss

Pathology Report

Clinical data: Left breast cancer, post NACT.
Nature Specimen: Left modified radical mastectomy specimen.

Gross Examination:
* Procedure: Left MRM measured 19x18x5 cm with covering skin ellipse measured 19x3 cm.
* Tumor extent: A greyish white firm tumor bed site.
* Size: 2.5 x2.5 x2 cm.
* Skin involvement: Nipple & areola were unremarkable grossly.
* Distance from margins: 1.5 cm away from deep margin.
* Lymph nodes number and size: 25 L.Ns up to 1.5x1.5x1cm.

Microscopic Examination:
* Histological type: Minimal microscopic remnant of invasive duct carcincma.
* Grade (differentiation+ nuclear pleomorphism+ mitosis): (3+2+) Gradel! .
- Maximal tumor diameter 0.6x0.6 cm
- Actual tumor cellularity: 3%, tumor necrosis index: 97%
* Ductal carcinoma in situ: 3%.
* Skin involvement: Free.
* Lymph node involvement:
* Resection Margins: Free.
* Histologic response to therapy: Partial pathological response.

Diagnosis:
Left breast cancer post NACT, MRM specimen:
Minmal microscopic remenats of Invasive duct carcinoma.
- Maximal tumor diameter: 0.6x0.6cm
- Free Resection Margins

Verified by :
Dr.Heba Hasan

غرفة رقم (4) – الدور الاول – مبنى رقم (987) – كورنيش النيل – مصر القديمة – القاهرة – مصر
Room No. (4) - First Floor-Building No. 987- Nile Corniche - Misr El Qadema - Cairo, Egypt

Patient Name	: الأستاذه / ناصرة احمد حسن	Request No.	: 13005369
Patient Nc.	: 26711150103969	Request Date	: 30/12/2023
Path No.	: 4608/23	Verification Date	: 15/01/2024
Sex	: Female	Financial Category	: اورام دار السلام - نفقة الدولة
Age	: 55 Y	Department	: Out Patient
Referreing Doc :		Registered by	: .marwa mohamed youss

Pathology Report

- Free nipple
- Positive Lymph nodes (14/25)
- Partial pathclogical response.
- Stage: ypT1b, ypN3
- RCB: 2.907, RCB class II .

Signature
Dr.Rana Fathy
Dr.Heba Hassan

Verified by :
Dr.Heba Hasan

غرفة رقم (4) - الدور الأول - مبنى رقم (987) - كورنيش النيل - مصر القديمة - القاهرة - مصر
Room No. (4) - First Floor-Building No. 987- Nile Corniche - Misr El Qadema - Cairo, Egypt

Patient Name	: الأستاذه / ناصرة احمد حسن	Request No.	: 13007861
Patient No.	: 26711150103969	Request Date	: 27/05/2023
Path No.	: 1602/23	Verification Date	: 06/06/2023
Sex	: Female	Financial Category	: اورام دار السلام - نفقة الدولة
Age	: 55 Y	Department	: Out Patient
Referreing Doc.	:	IHC No.	:
		Registered by	: .marwa mohamed yousse

IMMUNOHISTOCHEMISTRY REPORT

Clinical Data:
Left breast mass lesion
Nature of the specimen:
Tru cut biopsy collected 25-5-2023 (1:25PM)
Gross Examination:
Four greyish white tissue cores up to 1.5 cm . Totally submitted .
Sections prepared from paraffin block for immune- staining using Bench Mark ULTRA system for ER, PR,HER2, ER clone (1E2), PR clone (SP1), HER-2 clone (4B5), & Ki67 clone (30-9)

Microscopic examination:
- Histological type: Invasive duct carcinoma
- Grade (differentiation+ nuclear pleomorphism + mitosis): G II(3+2+1)
- Ductal carcinoma in situ: Focal (Solid pattern)
- IPT :

ER: Strong nuclear reaction in 80% of tumor cells (Allred score 8/8)
PR: Strong nuclear reaction in 80% of tumor cells (Allred score 8/8)
Her-2: No membrane reaction in tumor cells (score 0)
Ki 67: 25 % nuclear reaction in tumor cells

Diagnosis:
Left breast mass lesion , Tru cut biopsy:
- Invasive duct carcinoma grade II
- ER : Positive (8/8)
- PR : Positive (8/8)
- HER2 : Negative (score0)
- Ki 67: 25 % nuclear reaction in tumor cells

Verified by :
dr.esraa magdy



ناصره
احمد حسن احمد
٣ حارة الطرابيشي-ش السدالبراني
السيدة زينب - القاهره

٢٩٦٧١١١٥٠١٠٣٩٦٩

KY2822118



٢٩٦٧١١١٥٠١٠٣٩٦٩ ربة منزل ٢٠٢٥/٠١

انثى مسلمة

البطاقة سارية حتى ٢٠٣٢/٠١/٠٤

